

The Early Years Media Initiative for Children Revised Toddler Module: Building Social-Emotional Resilience



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Introduction

This chapter describes our scalable child-centred, socio-ecological programme to teach responsive caregiving to toddler staff who then implement the programme with children—and teach families these same approaches. We talk about our use of ongoing reflective support to the staff who then support parents and other caregivers to improve the capacity of all the adults to help children develop the skills needed to withstand the stresses of intergenerational conflict, and problem solve to build peaceful and sustainable futures. Our controlled trial demonstrated marked improvements in children’s social-emotional functioning, and decreased stress amongst staff and parents. We are now scaling the programme with plans for expansion nationwide and internationally.

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Programme content

The programme includes a 2.5-hour staff training session at the beginning of the academic year, covering 12 topics and using 12 micro training videos. The 12 videos total 30 minutes and the staff use the videos to convey the same ideas to parents. The curriculum is summarised on a one-page set of 12 cartoon panels that can be taped to a cabinet or refrigerator at home and in the classroom as a reference and reminder. The 12 topics include:

Section one:

Managing upset

1. The scenario: children under stress can grow up to be sad or angry, impeding their ability to engage in productive problem solving, eroding social cohesion, and creating a context for continued sectarian conflict.
2. Discussion of the usual reasons why children become dysregulated, using a mnemonic device 'HALT' (Hungry, Angry, Lonely, Tired) as a convenient tool to remember to think through those reasons rather than reacting to the surface behaviour. Hungry includes any kind of bodily discomfort or pain; Angry includes a range of frustrations; Lonely includes the range of difficulties from sadness to depression, to anxiety; Tired includes all the reasons why a child might be exhausted due to emotional, family, or community stress.
3. Culture begins before birth: infants are exposed to aspects of culture while still in the womb and they are born already knowing about their cultural group, and the importance of supporting family cultural identity.

4. Managing tantrums: training on two techniques utilising co-regulation for responding to upset children that build empathic connections. These involve the adult helping the child to become calmer and more able to participate in problem solving to figure out together why the child is having difficulty, and then addressing the difficulty.

Section two:

Building Problem Solving Resilience

5. Overview of the linkage between helping the child manage upset to allow her to build her skills for problem solving and how this leads to building a better, more peaceful society.
6. Wait: how to observe and assess a child as you begin to build resilience.
7. Join: How to connect with a child in a manner that gives you the ability to help them grow emotionally.
8. Build: How to engage in a meaningful flow of interaction and play that helps a child become a resilient, confident problem solver.

Section three:

Reflective Process – taking time to think and problem solve

Reflective Process helps the staff address challenges in supporting a child and their family by offering time to talk about the problems, their own reactions to these difficulties, and to think about ideas to address those problems. (Shamoon-Shanok, 2009). The characteristics of reflective process include the qualities a person brings to interactions with others, such as regulated, interested, emotionally engaged, authentic, and respectful. The back-and-forth interactions between the early

childhood professional and the staff is meant to model and experience a way of being that allows for skill development through affective interactive exchanges (Veloni, 2017). To accomplish this, the staff meet once a month during the year to talk about their work with the children. These meetings are facilitated by colleagues who have training in providing reflective support with a caring attitude, are genuine, non-authoritarian, and promote trust (Johnston & Brinamne 2012).

9. What not to do (1): Avoiding harsh treatment of others when trying to support them.
10. What not to do (2): Avoiding support that is often experienced as less than authentic.
11. What to do: Authentic, active (brave) listening. Brave means learning to tolerate difficult emotions that come up and committing to work together to try out solutions to problems.
12. The bigger picture: parallel reflective process from consultants to staff to parents to children. The programme is founded on the idea that we need to have a parallel support structure of staff supporting the responsive care of their children, and the staff themselves having the responsive support from colleagues.

These ideas become part of an everyday style of responsive caregiving. In the original Media Initiative for Children, the programme uses persona dolls from multiple cultural and social groups in the community with preschool children, to help the children and their families learn about and increase acceptance of people from those other groups. There are stories about the dolls depicting their backgrounds and television commercials utilising the characters in everyday situations and problems that preschool children face. In

the revised toddler module, the programme utilises persona dolls in developmentally meaningful ways for toddlers eg as peers helping to soothe when a child is upset to give the child the experience of cross-cultural empathy or joining in play-based interactions to build the child's experience and comfort playing with 'members' of other cultural communities.

Over the course of the academic year, the staff attend monthly reflective process groups led by either consultants or by selected staff members who have been trained in reflective process.

Programme monitoring involves measuring a minimum of ten percent of children to look at their social-emotional health, capacity for empathy, relationships with parents and caregivers, and parent and staff stress.

Inputs of the Revised Toddler Module

The inputs for the Revised Toddler Module include the identified communities that can benefit from early childhood programs and the process of staff development and oversight, which includes research and monitoring elements. The programme is designed to be implemented in the context of any existing childcare facility with minimal additional cost in time. The programme may save additional time and money because it leads to better regulated children who are more able to engage in everyday problem solving and which helps them to be more efficient and effective in everyday child and family function.

Activities of the Revised Toddler Module

The activities for the Revised Toddler Module include regular reflective support and training for staff, regular reflective support and modeling for parents by staff at drop off, pick up and in scheduled meetings during the course of the academic year. The team also conducts periodic outcomes monitoring.

Outputs of the Revised Toddler Module

The outputs for the Revised Toddler Module include monthly reflective meetings for staff over the academic term, weekly informal communication in the context of drop off and pick up, as well as periodic formal check-in meetings for parents with staff over the academic term. The staff also collect data on parent and staff experience and stress, staff and/or parent responsiveness, and child empathy and social-emotional function at the beginning and end of the academic year.

Outcomes of the Revised Toddler Module

The outcomes for the Revised Toddler Module include staff learning to seek and utilise reflective support for themselves, build on their use of responsive caregiving with children, experience less stress, and utilise reflective support and model responsive caregiving for parents. Parents will learn to seek and utilise reflective support for themselves, experience less stress, and use more reflective and responsive caregiving with their children. Children will become accustomed to and learn to utilise reflective and responsive caregiving from staff and parents, become more able to self-regulate (fewer behavioural challenges), become more empathic and more able in social-emotional problem-solving, ie resilience. The ultimate goal of the programme is that staff, parents, and children will have an increased acceptance of and appreciation of cultural and community diversity and differences. This can happen as a result of strengthening the resilience and empathy of the children, the responsiveness and problem solving of staff and parents, and the impact of using persona dolls.

Programme Development of the Revised Toddler Module of the Media Initiative for Children

Between 2014 and 2018, the team focused on programme development. The Early Years organisation and development team translated the DIR (Developmental, Individual differences, Relationship-based) model by Greenspan and Wieder (2006) into an Early

Childhood Education (ECE) programme. The organisation designed the programme to integrate into existing programmes. The programme included training for the ECE staff and reflective process meetings with the training team and ECE programme staff. The organisation also included a system of parallel reflective processes for the training team and the ECE programme staff.

Pilot Study

In 2019, the research team conducted a pilot study to determine whether the programme was feasible, acceptable, measurable, and promising. The study involved training staff for 2.5 hours daily for three days, and training parents for 30 minutes daily for three days. The study measured changes in staff responsiveness, parent and staff stress, child empathy, child regulation, and social-emotional development. The study showed the programme as feasible, acceptable, measurable, and very promising, with a steep increase in mastery of functional, and emotional developmental capacities in a group of 11 children aged 30-42 months.

Controlled Trials

Between 2019 and 2022, the research team attempted three rounds of year-long controlled trials to test whether the programme was effective. We first revised staff stress scales based on feedback to make them more relevant to staff, improved video and data collection using videographers when needed, and adding structured checklists to ensure better completion of data-gathering tasks. The trials were conducted in three annual phases, with the first starting in 2019. The trials involved staff training, parent training, and monthly reflective process groups for the test group as well as data collection for both the test and control groups. The test group consisted of children from the Rainbow SureStart toddler programme, and the control group came from the Splash SureStart toddler programme. SureStart programmes are publicly funded enrichment programmes for children from families from socioeconomically distressed communities. The ages of children

and parents, family incomes, and levels of education were comparable between the Rainbow and Splash groups. The first trial was difficult to initiate on time due to the organisational challenges of conducting a larger-scale, real-time research study with young children and their families. This trial was further impacted by the COVID-19 pandemic, and the focus of the research team pivoted to supporting families through the stress of lockdowns and illness. For the second trial in 2020–2021 the team revised the materials to allow for remote parent training by producing micro-videos of the programme principles. This trial was still difficult to complete in a satisfactory manner due to the stresses of the continuing pandemic. The third trial from 2021–2022 was far more successful and formed the basis for our data analysis.

Data Analysis as of May 2023

The data in the third and most complete trial showed that the groups were comparable in terms of the ages of children, parents, family income, education of parents, and mix of community identities. We again used the Social Emotional Growth Chart (SEGC) to measure the children’s progress. The SEGC showed that the children in the control group of 36 improved along the usual curve, and parents rated the children in mastery levels. The data showed that the children in the test programme, ie where the staff were trained and received the reflective process support meetings, began along the usual curve; however, much like in the pilot study, their social-emotional function improved at a statistically and clinically greater rate than the usual curve.

Controlled Trial Data

The SEGC is an adult report (parent and staff in this controlled trial) composed of a sensory processing section and eight distinct developmentally sequenced capacities (stages 1, 2, 3, 4a, 4b, 5a, 5b, 6). Sensory processing refers to the ability of the child to tolerate and respond to sensory information in the context of interactions. The social-

emotional capacities include being calm and regulated, emotionally connected with another person in the moment, able to communicate back and forth with another person in a meaningful way, able to use a flow of back-and-forth communication in problem solving and play, use of symbolic thinking in communication, and logical bridging of ideas in social communication, play, and learning. For our SEGC data, we analysed two separate outcomes: 1) SEGC total scores and 2) SEGC sensory processing scores.

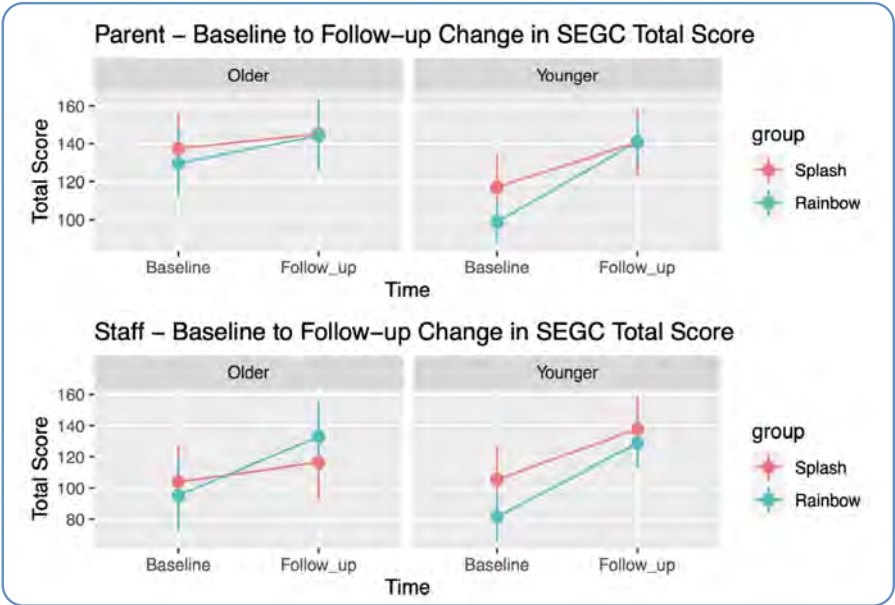
1) SEGC Total scores

The SEGC total scores are calculated by taking the sum of all completed sections from the sensory processing section to the last completed stage based on each child’s age. Since the completion of a particular SEGC stage is contingent on a child’s age, a handful of children in Rainbow (test) and Splash (control group) SureStart programmes were less than the age of 31 months at baseline, and therefore were not able to complete the final stage of the SEGC. All children at follow-up were able to complete the entirety of the SEGC, being that they were all either 31 months old or older. We decided to stratify our data based on whether each participant was old enough to complete all of the SEGC stages at baseline. This stratification was performed because children younger than 31 months old will have smaller total scores at baseline due to the fact that they were not old enough to complete the entire SEGC, making these participants more likely to have larger differences in total scores from baseline to follow-up just by virtue of being able to complete the entirety of the SEGC at follow-up. Ultimately, we decided to split our dataset for the parent and staff analyses of change in SEGC total scores from baseline to follow-up:

- 1) **Older Groups** = Children with ages greater than or equal to 31 months at baseline
- 2) **Younger Groups** = Children with ages less than 31 months at baseline

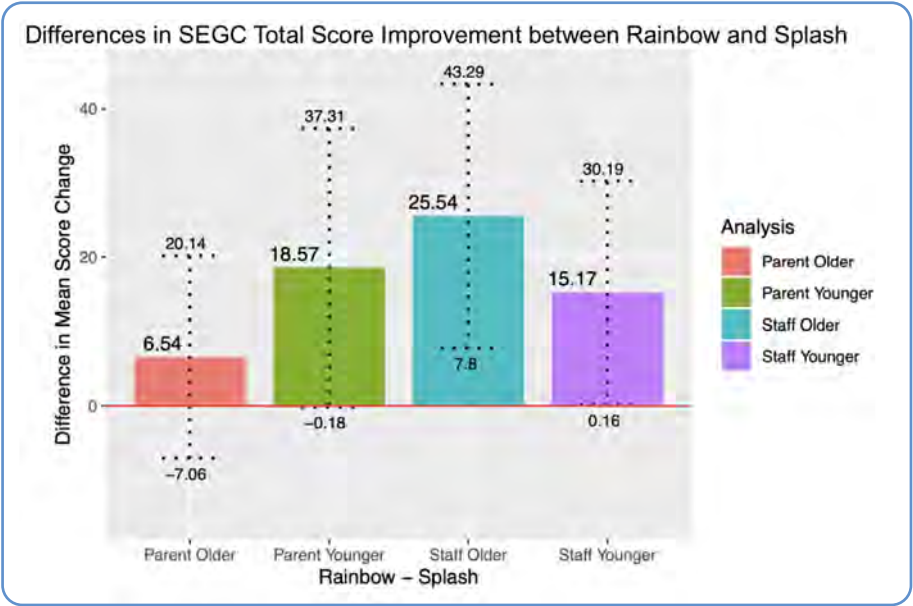
We compared baseline-to-follow-up changes in SEGC total scores between Rainbow and Splash to answer the question “Did Rainbow improve more than Splash?”.

The line graph below shows the change in parent and staff-reported SEGC total scores from baseline to follow-up.



Essentially, we analysed the differences in slopes of the lines representing Rainbow’s improvement and the lines representing Splash’s improvement to see if Rainbow had greater total score increases from baseline to follow-up.

Below are our findings after controlling for the child’s age at baseline and gender:



Parent Assessors

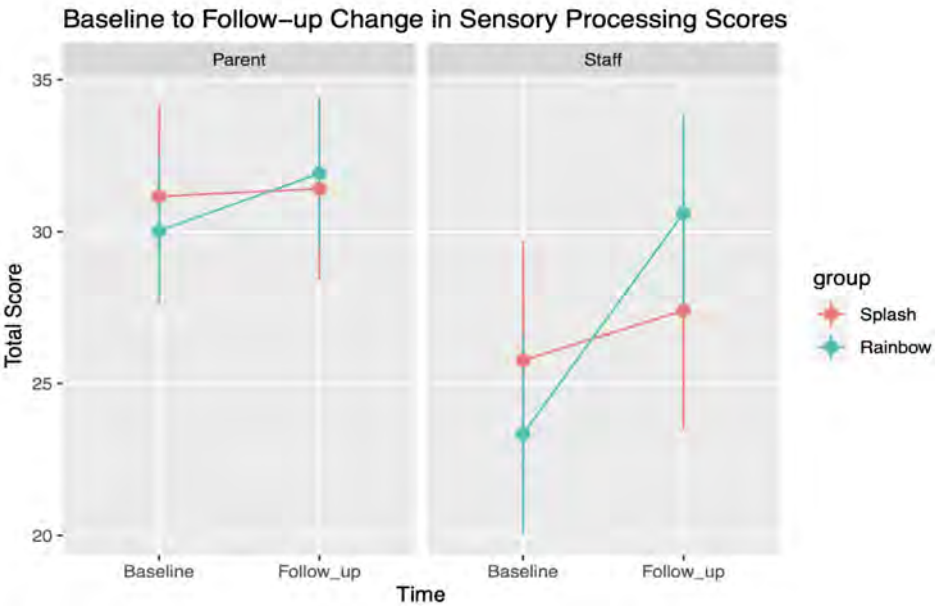
For the older and younger children, there were no significant differences between Rainbow (older: n = 17; younger: n = 19) and Splash (older: n = 16; younger: n = 8) in terms of improvement in SEGC total score from baseline to follow-up. However, both older and younger children in Rainbow showed greater improvement in SEGC total scores compared to their counterparts in Splash. On average, older children in Rainbow had a 6.54 (95% CI = -7.059, 20.14) greater improvement in total score compared to that of the older children in Splash between baseline and follow-up. Younger children in Rainbow also had an 18.56 (95% CI = -0.177, 37.308) greater improvement in total score than that of younger children in Splash between baseline and follow-up, and this finding was just shy of being statistically significant with a p-value of 0.052.

Staff Assessors

For the staff assessors, both the older and younger groups of children in Rainbow (older: n = 17; younger: n = 19) improved significantly more than their counterparts in Splash (older: n = 15; younger: n = 10). On average, the older Rainbow children had a 25.54 (95% CI = 7.796, 43.286; p = 0.006) greater improvement in total score compared to that of the older children in Splash between baseline and follow-up. In addition, younger children in Rainbow also had a 15.17 (95% CI = 0.156, 30.192; p = 0.048) greater improvement in total score than that of younger children in Splash between baseline and follow-up.

2) Sensory Processing

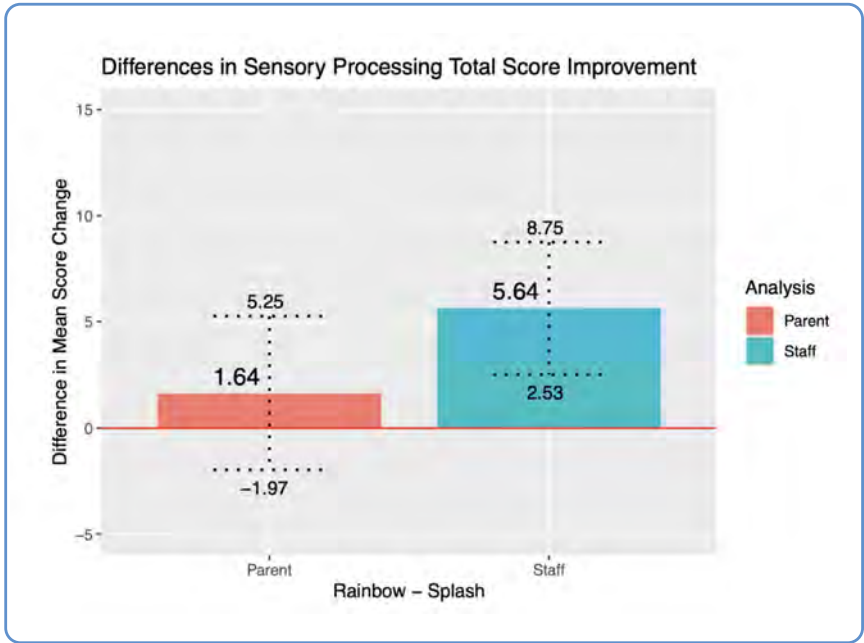
We did the same type of analysis to compare Rainbow (parents & staff: n = 36) and Splash (parents: n = 24; staff: n = 25) improvement in sensory processing scores from baseline to follow-up. The only difference is that the sensory processing section of the SEGC is not age-dependent, meaning that we were able to include all of the children in Rainbow and Splash without needing to stratify the data into age groupings.



Here are our results from the sensory processing score analysis after controlling for the child's baseline age and sex:

Parents: There was no significant difference in reported sensory processing score improvement between Rainbow and Splash. Yet, there was a nonsignificant trend favouring greater reported improvement in the Rainbow group, compared to Splash.

Staff: Children in Rainbow showed a significantly greater improvement in sensory processing scores from baseline to follow-up. On average, children in Rainbow had a sensory processing score improvement of 5.64 (95% CI = 2.53, 8.75; $p = 0.001$) points greater than that of Splash.



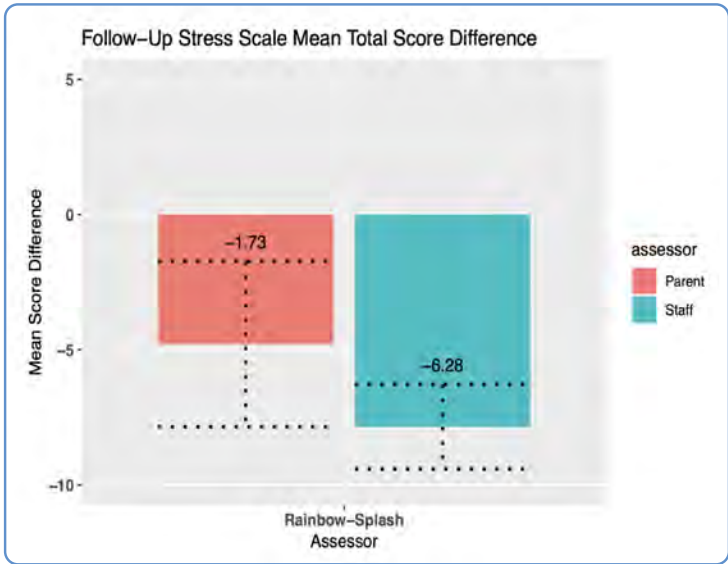
Stress Scale Analysis for Parents and Staff

To analyse the stress scales, positive items (e.g., "I am happy with . . ." or "I enjoy my . . .") were negated so that higher scores equate to higher stress levels.

Parents: After controlling for baseline stress scores, parents of children at Rainbow SureStart ($n = 35$) had significantly lower total stress scale scores at follow-up compared to that of Splash ($n = 25$). At follow-up, the difference between the mean stress scale total scores for Rainbow minus Splash was -4.79 (95% CI = -7.85, -1.73; $p = 0.003$), indicating that parents in Rainbow had significantly lower stress at follow-up compared to parents in Splash.

Staff: Similarly, after controlling for baseline stress scores, staff at Rainbow SureStart ($n = 7$) had significantly lower total stress scale scores at follow-up compared to that of Splash ($n = 9$). At follow-up, the difference between the mean stress scale total scores for Rainbow minus Splash was -7.84 (95% CI = -9.40, -6.28; $p < 0.001$), indicating that staff in Rainbow had significantly lower stress at follow-up compared with staff in Splash.

The bar graph below represents the difference in stress scale total scores between Rainbow and Splash at follow-up. The numbers present on top of the 95% confidence interval bars show the value of the lower confidence level not overlapping with zero, thus showing that, at follow-up, the estimated mean difference in stress scale total scores between Rainbow and Splash is not equal to zero.



Data analysis for the controlled trial is ongoing, focused on video samples of the interactions between children and staff at the beginning and end of the 2021-2022 academic year at Rainbow and Splash SureStart programmes. The additional findings are pending, including measurement of staff responsiveness and staff-child dyadic interactions. These will have implications for extending the scale up to embed video measurement into the scaled programme of parent and caregiver-child interactions. This would offer a more definitive assessment of the impact on child social-emotional growth and resilience as well as providing more feedback for parents and other caregivers to support their developmentally strengthening and resilience building relationships with their children.

Scale up

During the 2022-2023 academic year, we scaled the programme to nearly triple its size from the initial 36-child capacity to 91 children and families, with planned expansion to nearly double again in the coming year. The research team monitoring the outcomes for a portion of these children will check that their outcomes are similar to those in the pilot and controlled trials. The research team is also developing training of trainers' programmes for increasing the capacity for reflective support and staff training. We anticipate using innovative technology in the form of app-based video coding to gather more precise outcome data on the impact of the programme on parent-child relationships and social-emotional development.

Impacts of the Revised Toddler Module

The impacts of the Revised Toddler Module include staff and parents improving their skills and ability to collaborate in using responsive caregiving to support the development of their children. Better resilience will impact the children's overall health and mental health as they grow into adulthood and have children of their own. The expectation is that communities receiving the programme will have a greater capacity for current and future sustainable inclusion, collaboration, and peace. Further research will help to clarify whether these long-term impacts are achieved. The programme is implemented in the context of any existing childcare facility with minimal additional cost in time and potentially saving time and costs because it leads to more efficient

and effective child and family function. The number of children and families impacted is easily scalable, limited only by the capacity of the research-management team in time for training and implementation.

What are the lessons to date?

The programme has evolved over the period from Pilot phase to Random Control Trial. This item will consider the lessons learnt in two aspects: research development phase and programme implementation no longer linked to research trials.

It needs to be acknowledged that the participants have provided the information which has allowed the programme to be developed to this current point. It has reached a degree of agreement that it is meeting the needs of the communities it has been delivered in during the period September 2022 to June 2023.

Research Development phase

During the phase March 2019 to June 2022 the programme was gathering data from families, staff and the children linked to the impact of the programme on social-emotional development with a specific focus on emotions and behavioural responses.

The research team during this period addressed several emerging elements relating to:

- the logistics of implementing the programme requirements
- utilising the programme for two-year-olds without requesting additional time or additional workload from the staff and families
- building the knowledge of the staff and how to show evidence of impact
- how to engage families in the programme

The following issues were identified during this phase which were then either amended, replaced or removed.

Data gathering

The timeline for baseline data was altered to reflect the personalised context of the SureStart.

Initially the Revised Toddler Module required families to complete the forms whilst attending an in person 30-minute training session over 3 consecutive sessions.

The reasoning for this approach was to ensure that there were familiar staff from the SureStart on hand to support parents with completing the forms. This worked well for the initial Pilot group but became problematic as the programme moved to a much larger Random Controlled Trial phase. This phase highlighted the unique delivery of services the SureStarts utilise within their own service delivery models.

At the same time, Covid-19 interrupted the service delivery of the SureStarts and impacted on the type of support provided for data form completion. The learning from this experience was the recognition that the SureStart settings were capable of collecting and providing the data but the manner in which this was achieved could be left to the individual SureStart, as long as they worked within clear guidelines. Amending this expectation of the programme respects the unique service delivery model which each SureStart site develops to reflect the current needs of each cohort of families and children.

Settings have their own autonomous and contextualised methods of family engagement which gives them ownership of building the relational aspect of the Revised Toddler Module programme. In this way the programme acknowledges the nature of each setting, recognises its capacity to conduct this aspect of data gathering, and provides the flexibility for settings to determine the way this will be completed.

Training for parents

Initially the programme design required parents to attend, in person, 30 minutes of training over 3 subsequent sessions. These were planned to be delivered straight after the children were entered into the SureStart programme two-year-old sessions. The initial pilot, which included 12 families, was very positive in attendance numbers and the level of engagement by the participants.

As the programme moved into the Random Control Trial phase where the family numbers increased over two different SureStart projects, parent training became challenging. The team was not able to conduct in-person training over three consecutive sessions as this did not align to the needs of the families of these cohorts. They were unable to commit to attending sessions at drop-off. The onset of the Covid-19 pandemic provided an opportunity for this element to be altered. The move to using video as the method for delivering the parental training proved to be a positive alteration for the overall programme. As a result, three training videos were created for parents to access at their own leisure which would be followed up by staff during their normal parental engagement points.

The learning from this challenge is that this type of media has given SureStart settings and parents the autonomy to achieve training input which reflects their individual cohort needs. This method allows parents the opportunity to access unlimited viewing of the training material thus increasing knowledge and confidence of the programme. It also means that parents can easily share content with other family members, enabling a wider circle of informed adults around the child.

Specific vocabulary used within the training programme

Part of the training programme pays attention to the history of Northern Ireland, how this affects communities' past and present, the impact of this legacy and how the programme addresses transgenerational trauma for communities through an ECD project.

The use of the word "conflict" was consciously used within the information and consent forms to indicate the aim of working toward more peaceful social outcomes over the long term as the toddlers grow into adulthood.

As an outcome, a range of responses from participating parents and staff during both the research phase and programme implementation were acknowledged.

During the research phase, the use of the word conflict produced a strong emotional response in a small group of parents. The dialogue with parents following this response identified a number of points such as:

- Northern Ireland is no longer in conflict
- Northern Ireland did not experience conflict
- discomfort in talking about what conflict means
- learned behaviour to not discuss a controversial topic.

As a result, the programme has removed the word conflict from its information sheet but continues to address the issue through the training material. This is not to avoid the issue but to recognise that people and communities have ranging responses to the written word demonstrating significant effect for some participants.

The programme wishes to support the building of positive relationships between family and setting and the use of the word seemed to raise barriers to that happening at the outset.

As a consequence of this experience, the programme has moved the sensitive subject to within the training where it can be framed within relationship dialogue. This has proved to have engagement of an educational type of response and support more productive integration of the emotions that are tied to the conflict.

The programme does not avoid the responsibility for addressing the subject of conflict. In fact, staff have requested further input on this subject, wishing to develop the following:

- confidence in starting conversations of this sensitive nature
- educating themselves on the impact that conflict brings to person, community and a workforce in a relational model of service
- building their own personal skills when dealing with a perceived controversial matter.

Regarding the families, one of the guiding principles of the programme is the necessity for establishing strong, trusting relationships between the family and staff members. This will then allow for further opportunities to address the matter within authentic relational moments which have the potential to enable people and communities to connect with the subject, reducing anxiety and dysregulation.

Staff member development

A fundamental aspect of the programme is to develop the skills and knowledge of the staff members to understand how to better manage and respond to the actions and responses from very young children, linked to emotions.

The aim is to provide training which helps the staff members to be knowledgeable and skillful in understanding the uniqueness of what it is like to be two years old. Thus, building informed connections supporting co-regulation and fully appreciating the importance of their role not only in that moment but within the journey to self-regulation for the child.

A key element to achieving this outcome with staff is the regular implementation of reflective process sessions. These sessions are designed to be facilitated by a nominated participant who is known to the staff member and has established trust with the group. The participants bring a concern or conundrum relating to a child's behaviour, learning development or family engagement for collective problem-solving.

As a result of implementing the programme, the SureStart Project managers commented on the improved quality and depth of dialogue by staff around child and family needs.

The key findings are:

- Thinking is now more holistic and explores the reasons behind behaviours. This is evidence of the application of training on a day-to-day basis within the practice
- Conversations are richer in depth of thinking with improved confidence to talk to a range of child development professionals as well as parents
- Thinking has moved to a robust implementation of a Benefit Model Approach, ie where the positives of the child and/or situation are acknowledged before any areas of concern are considered
- Improved collaboration and sharing of good practice within and across teams
- Opportunities to have exposure to others' viewpoints, enabling awareness and acceptance of different opinions and suggestions
- Improved confidence when interacting with parents about their child's needs which is based on sound knowledge and understanding, supported through the training and reflective sessions
- Improved staff member confidence and independence to appropriately manage big emotions by very young children, enabling a suitable environment which encourages age-appropriate development
- Improved understanding of how to build authentic relationships, empowering co-regulation and self-regulation.

As a result of the multiyear efforts of the research team, the programme has considered how to provide training for facilitators at little or no cost, with the outcome of building the capacity of the staff team to take this role. The programme continues to develop in this area.

Challenges and Next Steps

There is now empirical evidence which identifies that the RTM-MIFC programme has the capacity to develop into a reliable programme that can be utilised within any childcare programme, addressing the needs of two-year-olds.

However, getting to this point has not been without challenge. The following description recognises the current status of the programme, and what needs must be addressed to produce a fully funded well-resourced programme.

Developing a local multi-disciplinary team to support staff member reflective sessions across Northern Ireland

Part of the success of the reflective sessions have been the inclusion of a range of expertise which has included:

- Child Psychiatrist
- Pediatric Occupational Therapist
- Media Psychologist
- Occupational Therapist/PhD in Infant and Early Childhood Development
- Mental Health Clinician & PhD Candidate in Infant and Early Childhood Development
- Occupational Therapist
- Early Years Teacher

This has enabled the group to hear from different disciplines, which has enhanced the quality of the problem-solving dialogue, whilst providing confidence for the staff that the input and outcomes are based on sound judgement and contribution.

The challenge for this issue is exploring opportunities for funding and professional input which will enable the programme to build its capacity to reach more settings, support staff member professional development, and establish the longevity of the support through Reflective Process sessions.

Employing a team of trainers to increase programme implementation across Northern Ireland and beyond

Consideration must be given to enabling this programme to achieve more than the current capacity of the team. The term “scale up” is used in this instance to explain the expansion of the programme to reach more settings, families, and staff.

An initial consideration when addressing scale up is to develop and determine a team of trainers to increase programme implementation across Northern Ireland and establish its fidelity. Also, we want to consider a scale up plan which will connect team capacity with increasing numbers of settings and the capacity for funding to allow this programme to be implemented.

Addressing legacy and language associated with conflict

As mentioned previously, the programme does not evade its responsibility relating to tackling this sensitive issue but acknowledges the difficulty it poses for all participants. The challenges faced in addressing this matter are complex. The next steps are to ensure there is a mechanism for every trainer to handle this matter with sound knowledge, keen awareness, confidence and skill.

It must be acknowledged that there will always be work within the peacebuilding arena. This element of the programme must not stand still or even look back, but forge ahead to support the ability of all members of a society to enter into dialogue and learning.

Continual development of employing multi-media as a method to engage programme participants and enrich assessment

The programme has adapted its training delivery model for parents and families from in-person to video because of the Covid-19 pandemic. The programme continues to review the use of media as both a training and a recording tool. The programme initially required that staff video children on entry and at exit (10-minute clips) so their interactions could be assessed by the research team. Now as the programme is considering expansion to additional childcare settings, it is beyond the capacity of the current research team to monitor even 10% of the children participating.

Currently there is a consideration about how this could be accomplished:

1. training staff to code their own videos using an IOS app
2. training parents to use videos of their own children and to code what they see using the same app
3. the confidentiality of videos and other data is a continuing consideration, particularly related to cyber security.

Conclusion

The Revised Toddler Module of the Early Years Media Initiative for Children is an innovative programme that focuses on improving the social-emotional resilience and problem-solving of young children, preparing them for a future of more enduring and productive social cohesion.

Contributors:

We wish to acknowledge the many years of dedicated work by our research and implementation team, without which this programme would not be possible. Each of the contributors listed below performed critical tasks directly related to the programme and this chapter.

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References



Clough, M., Nilsen, S., & Feder, J. (2013). Inter-rater reliability in video coding of engagement between caregivers and children outside of laboratory settings. Unpublished.

Connolly, P., Smith, A., Kelly, B., & Miller, S. (2002). Too Young to Notice? The Cultural and Political Awareness of 3-6 Year Olds in Northern Ireland. Ulster University.

Feder, J., & Kernohan-Neely, Z. (2023, June). Shaping the future when the future is uncertain. Early Years International Conference, Belfast.

Feder, J. (2021). Wait - Join - Build: Implementing A Regional Responsive Caregiving Program to Improve Resilience in Young Children Impacted by Conflict. World Association of Infant Mental Health, Adelaide, Australia.

Feder, J. (2021). Can limited staff training and reflective practice change resilience, parental behavior, and developmental trajectories in toddlers impacted by armed conflict? The Revised Self-Regulation Toddler Module of the Media Initiative For Children. World Association of Infant Mental Health, Adelaide, Australia.

Feder, J. (2019). Developmental Disorders, Diagnosis and Tools. Paper presented at the International conference on ECD Children, hope, and peace a local and global perspective, An-Najah National University, Nablus-Palestine.

Feder, J. (2016). Are We Connected? How to Use Everyday Video to Track Developmental Progress in Social Communication. Autism Europe Conference, Edinburgh.

Feder, J. (2016). Supporting Resilience in Young Children Living in Regions Impacted by Conflict. American Academy of Child and Adolescent Psychiatry Annual Conference, New York, NY.

Fayyad JA et. al. (2010) Dissemination of an evidence-based intervention to parents of children with behavioral problems in a developing country. *Eur Child Adolesc Psychiatry*. 2010 Aug;19(8):629-36

Greenspan, S., & Wieder, S. (2006). *Infant & Early Childhood Mental Health: A Comprehensive Developmental Approach to Assessment and Intervention*. Paul H. Brookes Publishing Co..

References. Continued.

- Greenspan, S., & Shanker, S. (2002). *Toward a Psychology of Global Interdependency: A Framework for International Collaboration*. York University.
- Harris, K. (2021). *A Study of Empathic Responses in Preschool-Age Children Using a Kind Test Method and EmQue Test Scores*. Fielding Graduate University. ProQuest Dissertations Publishing (28543411).
- Heffron, M. (2011). *Reflective Supervision and Leadership in Infant and Early Childhood Programs*. Gryphon House.
- Johnston, K., & Brinamen, C. F. (2012). The consultation relationship—From transactional to transformative: Hypothesizing about the nature of change. *Infant Mental Health Journal*, 33(3), 226-233.
- Miller, B., & Sprang, G. (2017). A Components-Based Practice and Supervision Model for Reducing Compassion Fatigue by Affecting Clinician Experience. *Traumatology*, 23(2), 153-164.
- Morrow, D., et al. (2019). *Sectarianism in Northern Ireland: A Review*. Ulster University. National Child Traumatic Stress Network nctsn.org.
- Osten, E. (2023) *Mother-Child Connectedness in Autistic and Neurotypical Preschoolers: Connection Coder Validity and Reliability*. Fielding Graduate University ProQuest Dissertations Publishing (30417990).
- Perry, B. And Winfrey, O. (2021) *What Happened to You?* Flatiron Books.
- Sandbank, M. et. al. (2020) Project AIM: Autism intervention meta-analysis for studies of young children. *Psychological Bulletin*, 146(1), 1-29.
- Silberg, J., & Schwartz, C. (2017). Expanding our Toolkit through Collaboration: DIR/Floortime and Dissociation-Informed Trauma Therapy for Children. *Frontiers in the Psychotherapy of Trauma and Dissociation*, 1(1), 45–64.
- Solomon, R. et. al. (2014). PLAY Project Home Consultation intervention program for young children with autism spectrum disorders: A randomized controlled trial. *Journal of Developmental and Behavioral Pediatrics*, 35(8), 475.
- Tomlin, A. M., Weatherston, D. J., & Pavkov, T. (2014). Critical components of reflective supervision: Responses from expert supervisors in the field. *Infant Mental Health Journal*, 35(1), 70-80.
- Tronick, E. (2007). *The Neurobehavioral and Social-Emotional Development of Infants and Young Children*. W.W. Norton & Co.
- Veloni, G. (2017). *Effects of Online Reflective Supervision/Practice Groups*. Fielding Graduate University.